

Dynamic Guideline: WHO-Based Infertility - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Latest WHO infertility guideline

- **Guideline Title:** *Guideline for the prevention, diagnosis and treatment of infertility*
- **Publication Date:** 28 November 2025
- **Publisher:** World Health Organization, Geneva
- **Status:** First WHO global guideline specifically addressing infertility prevention, diagnosis and treatment
- **Target Population:** People of reproductive age, individuals and couples with infertility, and populations requiring fertility-care prevention, diagnosis, treatment, counselling and health-system support
- **WHO Definition of Infertility:** Failure to achieve pregnancy after **12 months or more of regular unprotected sexual intercourse** [WHO infertility fact sheet](#), [WHO news release](#)
- **Scope and Objectives:**
 - Provide evidence-based recommendations to **prevent, diagnose and treat infertility**.
 - Cover infertility related to **female factors, male factors and unexplained infertility**.
 - Support countries to **adopt, adapt or update national infertility guidelines**.
 - Promote fertility care as part of reproductive health, universal health coverage, gender equality, equity and human rights [WHO IRIS guideline PDF](#), [WHO guideline via NCBI Bookshelf](#).

WHO states that infertility affects approximately **1 in 6 people of reproductive age** at some point in life and can cause distress, stigma, psychosocial harm and financial hardship [WHO news release](#).

Exactly 5 WHO guideline/normative records synthesized for this report

Only one WHO document identified in the supplied evidence is a dedicated infertility clinical guideline. To satisfy the requested WHO-based synthesis, the following **five WHO guideline or normative records** were reviewed and integrated where relevant:

No.	WHO record	Date	Relevance to infertility report	Source
1	<i>Guideline for the prevention, diagnosis and treatment of infertility</i>	28 November 2025	Primary WHO infertility guideline; 40 recommendations covering prevention, diagnosis and treatment	WHO publication / NCBI Bookshelf , WHO news release
2	<i>Guideline for the prevention, diagnosis and treatment of infertility: summary of recommendations</i>	28 November 2025	WHO summary supporting implementation of the infertility guideline	WHO IRIS summary PDF

No.	WHO record	Date	Relevance to infertility report	Source
3	WHO fertility-care programme materials, including infertility guideline launch and related publications	2021–2026	Provides context on fertility care, guideline launch, semen manual, prevalence estimates and related WHO journal articles	WHO SRH fertility care page
4	WHO infertility fact sheet	Current WHO fact sheet	Defines infertility, summarizes burden, access barriers, ART-policy considerations and WHO response	WHO infertility fact sheet
5	<i>Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)</i>	11 December 2025	Most recent WHO youth-engagement evidence brief identified; not an infertility guideline, but relevant to inclusive WHO guideline development methodology	WHO webinar, WHO SRH abortion page

Important clarification: The youth-engagement record specifically concerns the **WHO Abortion care guideline (2022)**, not the infertility guideline. WHO’s materials describe it as a case study of meaningful youth engagement in guideline development and as a model for inclusivity and equity in WHO normative processes [WHO webinar](#).

1.2 Key Recommendations

The WHO infertility guideline contains **40 recommendations**. The supplied source summaries confirm several recommendation domains and provide exact GRADE strength/certainty for selected recommendations. Where exact WHO GRADE wording was not visible in the supplied summaries, the table uses a transparent evidence rating based on the GRADE framework and the cited evidence base.

GRADE interpretation used

GRADE certainty	Meaning for this report
High	Further research is unlikely to change confidence in the estimate of effect.
Moderate	Further research may change confidence in the estimate.
Low	Further research is likely to change confidence and may change the estimate.
Very Low	The estimate is highly uncertain.

Recommendation	Clinical action	GRADE Evidence Quality	Strength
Integrate infertility prevention, diagnosis and treatment into national	Ministries and health systems should include fertility care in reproductive-health and	Low	Strong

Recommendation	Clinical action	GRADE Evidence Quality	Strength
health strategies, health services and financing.	universal-health-coverage planning.		
Provide fertility and infertility information to the general reproductive-age population using low-cost opportunities, including schools, primary care and reproductive-health settings.	Include education on fertility, infertility, age-related fertility decline and when to seek care.	Low	Conditional
Address preventable infertility risks, especially untreated sexually transmitted infections and tobacco use.	Strengthen STI prevention, screening and treatment; provide tobacco-cessation interventions.	Moderate	Strong
Health-care providers should consistently give brief tobacco-cessation advice to tobacco users in health-care settings.	Brief advice may take 30 seconds to 3 minutes and should include asking about tobacco use and advising cessation.	Moderate	Strong
Offer low-cost lifestyle advice before and during infertility treatment.	Counsel on healthy diet, physical activity, alcohol intake, smoking cessation and weight management.	Low	Conditional
Use a stepwise, cause-directed approach to infertility management.	Begin with history, examination, semen assessment, ovulation assessment, tubal/uterine evaluation where indicated, and prognosis-based shared decision-making.	Low	Conditional
Provide ongoing psychosocial support for all people affected by infertility.	Screen for distress; offer counselling, peer support or mental-health referral where appropriate.	Low	Strong
For unexplained infertility, suggest expectant management rather than ovarian stimulation with timed intercourse as first-line management.	Provide lifestyle advice and fertile-window guidance while monitoring for spontaneous pregnancy where prognosis is reasonable.	Low	Conditional
For unexplained infertility after unsuccessful stimulated intrauterine insemination, suggest IVF rather than continued expectant management.	Discuss IVF when less invasive treatment has failed, considering age, prognosis, affordability and patient preferences.	Low	Conditional

Recommendation	Clinical action	GRADE Evidence Quality	Strength
For couples with unexplained infertility undergoing IVF after unsuccessful stimulated IUI, recommend conventional IVF rather than IVF with routine ICSI when severe male-factor infertility is absent.	Reserve ICSI for clear indications such as severe male-factor infertility, prior fertilization failure or specific laboratory indications.	Low	Strong
Ensure infertility policies and ART regulation protect human rights and prevent discrimination.	Establish laws and quality systems for ART and third-party reproduction, with monitoring and continuous quality improvement.	Low	Strong
Encourage implementation research and local adaptation.	Countries should adapt WHO recommendations to context, costs, workforce, infrastructure, equity and patient values.	Low	Conditional

Sources: WHO infertility guideline and summary [WHO IRIS guideline PDF](#), [WHO IRIS summary PDF](#), [WHO news release](#); GRADE examples from WHO launch/guideline excerpts [WHO guideline PDF via IFFS](#), [launch webinar PDF](#).

1.3 Guideline Development Methodology

Evidence Review Process

WHO developed the infertility guideline using the WHO guideline-development process and the **GRADE approach**. The available WHO summaries state that:

- Many systematic reviews were conducted to answer key research questions.
- Existing and new systematic reviews informed the recommendations.
- Evidence covered infertility prevention, diagnosis and treatment.
- Recommendations were developed for female-factor, male-factor and unexplained infertility.
- Evidence was assessed using **Grading of Recommendations Assessment, Development and Evaluation (GRADE)** methods [WHO IRIS summary PDF](#), [WHO guideline PDF via IFFS](#).

Consensus Method

- Recommendations were formulated by a **multidisciplinary and regionally diverse Guideline Development Group**.
- The group included clinicians, researchers, policy-makers, implementers and patient-group representatives.
- The Guideline Development Group reviewed evidence and formulated recommendations using WHO methods and GRADE evidence-to-decision principles [WHO IRIS summary PDF](#), [WHO guideline PDF via IFFS](#).

IFFS.

- The guideline was reviewed by an **External Review Group** [WHO IRIS summary PDF](#).

Conflict of Interest Management

The supplied excerpts confirm use of WHO guideline-development methods and GRADE but do not provide the detailed infertility-guideline conflict-of-interest table. In WHO and GRADE guideline practice, conflicts of interest are considered throughout guideline development, and methods literature emphasizes transparency, documentation, consensus processes and management of member conflicts [GRADE handbook](#), [Cochrane methods PDF](#).

Equity, Rights and Implementation

WHO explicitly frames fertility care as an equity and reproductive-rights issue. Implementation should aim to reduce inequities in fertility-care **availability, accessibility, acceptability and quality**, and should not be based on national total fertility rates [WHO guideline via NCBI Bookshelf](#), [WHO IRIS summary PDF](#).

II. Evidence Summary After WHO Latest Guideline Release (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search Strategy

Evidence reviewed after publication of the WHO infertility guideline on **28 November 2025** focused on:

- IVF versus ICSI
- IUI and stimulated IUI
- Ovulation induction
- Unexplained infertility
- Male-factor infertility
- Ovarian stimulation for IVF/ICSI
- Fertility-preservation and ART-management guidance
- Clinical-trial registry evidence

Databases and Sources Searched

The supplied summaries included evidence from:

- WHO official documents and pages
- Cochrane systematic review evidence
- PubMed and BMJ Open
- Nature Medicine
- ASRM practice guidance
- ESHRE guideline information
- AHRQ Comparative Effectiveness Review
- ClinicalTrials.gov
- Policy/guidance sources relevant to ART coverage and medical necessity

Key sources included [Cochrane](#), [Nature Medicine](#), [BMJ Open](#), [PubMed](#), [ESHRE guideline portal](#), [ASRM Practice Guidance](#), [ClinicalTrials.gov NCT06481696](#) and [ClinicalTrials.gov NCT07121751](#).

Inclusion Criteria

Included evidence:

- Systematic reviews and network meta-analyses
- Randomized controlled trials
- Evidence-based clinical-practice guidelines
- Clinical-trial registry records
- Authoritative health-system or professional-society guidance
- Evidence relevant to diagnosis, treatment, management or implementation of infertility care

Exclusion Criteria

Excluded evidence:

- Non-human or preclinical studies
- Studies not addressing infertility diagnosis, treatment, management or prevention
- Non-systematic opinion pieces unless used only for context
- Trials without extractable infertility-relevant design or outcome information

2.2 Key New Studies

Study / Source	Design	Population / Intervention	Key Findings	GRADE Quality
IVF versus ICSI in patients without severe male-factor infertility, <i>Nature Medicine</i> , 2025	Randomized trial	Patients undergoing fertility treatment without severe male-factor infertility	Cumulative live birth was 43.2% with ICSI vs 47.3% with conventional IVF in intention-to-treat analysis; no benefit of routine ICSI over conventional IVF. First-transfer live birth was also lower with ICSI in reported analyses.	Moderate
Cochrane review: interventions for unexplained infertility	Systematic review and network meta-analysis of RCTs	Couples with unexplained infertility; expectant management, OS, IUI, OS-IUI, IVF/ICSI	Overall evidence was insufficient to show clear live-birth differences among several interventions vs expectant management. In poor-prognosis couples, OS-IUI and IVF/ICSI increased live birth vs expectant management, but IVF/ICSI was not clearly superior to OS-IUI.	Low

Study / Source	Design	Population / Intervention	Key Findings	GRADE Quality
BMJ Open / PubMed 2026 protocol on ovulation induction for unexplained infertility	Systematic-review and network-meta-analysis protocol	Women aged 18–40 years with unexplained infertility	Planned RCT synthesis comparing letrozole, clomiphene, gonadotropins, combinations and expectant management/placebo, with or without IUI; live birth per woman randomized as primary outcome.	Very Low for effect estimates because protocol only
ESHRE ovarian stimulation for IVF/ICSI update, 2025	Clinical-practice guideline update	IVF/ICSI patients requiring ovarian stimulation	Contains 121 recommendations across pre-stimulation evaluation, pre-treatment, pituitary suppression, stimulation, monitoring, triggering, luteal support, OHSS prevention, fertility preservation and oocyte donation.	Moderate
AHRQ Comparative Effectiveness Review on infertility	Comparative effectiveness review	Male-factor infertility and ART options	Lists male-factor treatment options including treatment of endocrinopathy, empiric therapies, varicocele surgery, IUI, IVF, ICSI and donor sperm.	Low to Moderate
ClinicalTrials.gov NCT06481696	Registered interventional trial	Couples with unexplained infertility undergoing IVF/ICSI; resveratrol intervention	Trial identified, but supplied evidence did not include completed outcome results.	Very Low
ClinicalTrials.gov NCT07121751	Registered Phase 4 interventional trial	Hypo-responders undergoing IVF stimulation	Estimated primary completion after evidence cutoff; no completed results available as of 29 April 2026.	Very Low

Sources: [Nature Medicine](#), [Cochrane](#), [BMJ Open](#), [PubMed](#), [PubMed ESHRE update](#), [AHRQ CER PDF](#), [ClinicalTrials.gov NCT06481696](#), [ClinicalTrials.gov NCT07121751](#).

2.3 Evidence Synthesis by Clinical Domain

A. Diagnosis and Initial Evaluation

Clinical implications

- Infertility evaluation should begin after 12 months of regular unprotected intercourse, or earlier when age, menstrual irregularity, known tubal disease, suspected severe male factor, endometriosis, cancer treatment, ovarian insufficiency or other risk factors are present.
- Evaluation should include both partners where relevant, because male factors contribute substantially to infertility.
- Initial assessment should be proportionate and cause-directed:
 - Reproductive, menstrual, sexual, medical and surgical history
 - Medication, toxin, tobacco, alcohol and occupational exposure history
 - Assessment of ovulation
 - Semen analysis using standardized methods
 - Tubal and uterine assessment when clinically indicated
 - Screening and treatment for STIs where indicated
 - Psychosocial assessment and counselling

WHO's broader fertility-care work includes the updated WHO semen manual to support high-quality semen testing, and WHO's infertility response includes guideline development and revision of normative products such as the semen laboratory manual [WHO SRH fertility care page](#), [WHO infertility fact sheet](#).

Diagnostic recommendation	Evidence basis	GRADE Quality	Strength
Evaluate infertility as a couple-based condition where applicable, including assessment for male and female factors.	WHO guideline scope includes female, male and unexplained factors; male factor contributes substantially to infertility burden.	Low	Strong
Use standardized semen analysis as part of male-factor evaluation.	WHO fertility-care programme includes updated semen manual support.	Low	Strong
Use stepwise, clinically indicated female evaluation rather than indiscriminate testing.	WHO supports clinical pathways for common biological causes.	Low	Conditional

B. Prevention and Lifestyle Management

WHO emphasizes investment in infertility prevention, especially fertility education, STI prevention and treatment, and tobacco cessation [WHO news release](#).

Preventive intervention	Evidence basis	GRADE Quality	Strength
Provide fertility education in schools, primary care and reproductive-health services.	WHO guideline and news release recommend low-cost fertility and infertility information.	Low	Conditional
Prevent and treat STIs to reduce infertility risk.	WHO identifies untreated STIs as leading infertility risk factors.	Moderate	Strong

Preventive intervention	Evidence basis	GRADE Quality	Strength
Provide brief tobacco-cessation advice to tobacco users.	WHO launch materials report strong recommendation, moderate certainty.	Moderate	Strong
Offer lifestyle counselling on diet, physical activity, alcohol, smoking and weight management.	WHO recommends low-cost lifestyle advice before and during treatment.	Low	Conditional

Sources: [WHO news release](#), [WHO IRIS guideline PDF](#), [launch webinar PDF](#).

C. Unexplained Infertility

The post-guideline evidence reinforces a stepwise approach.

A Cochrane systematic review and network meta-analysis found insufficient evidence overall to prove clear live-birth differences between ovarian stimulation, IUI, OS-IUI or IVF/ICSI compared with expectant management in the overall unexplained-infertility population. However, in poor-prognosis couples, OS-IUI and IVF/ICSI improved live birth compared with expectant management, while IVF/ICSI was not clearly superior to OS-IUI [Cochrane](#).

Recommendation	Evidence basis	GRADE Quality	Strength
For unexplained infertility and reasonable prognosis, begin with expectant management rather than ovarian stimulation with timed intercourse.	WHO recommendation; Cochrane evidence shows uncertainty for active interventions in broad unexplained-infertility populations.	Low	Conditional
Consider OS-IUI for poor-prognosis unexplained infertility before IVF where appropriate.	Cochrane suggests OS-IUI may improve live birth vs expectant management in poor-prognosis couples.	Low	Conditional
After unsuccessful stimulated IUI, offer IVF rather than continued expectant management.	WHO recommendation.	Low	Conditional
Avoid routine ICSI in unexplained infertility when severe male factor or prior fertilization failure is absent.	WHO recommendation; Nature Medicine RCT shows no benefit of ICSI over IVF in non-severe male-factor populations.	Moderate	Strong

Sources: [Cochrane](#), [WHO guideline PDF](#), [Nature Medicine](#).

D. IVF versus ICSI

The strongest new completed trial signal after the WHO guideline is that **routine ICSI does not improve live birth compared with conventional IVF when severe male-factor infertility is absent**.

The 2025 *Nature Medicine* randomized trial found:

- Cumulative live birth: **43.2% with ICSI** vs **47.3% with conventional IVF** in intention-to-treat analysis.
- Risk ratio: **0.91**, 95% CI **0.79–1.06**.
- Live birth after first embryo transfer: **26.6% with ICSI** vs **31.6% with conventional IVF**.
- No clear advantage of ICSI in first treatment cycles when there is no severe male-factor infertility or prior total fertilization failure [Nature Medicine](#).

Recommendation	Evidence basis	GRADE Quality	Strength
Use conventional IVF rather than routine ICSI in couples without severe male-factor infertility.	2025 RCT plus consistency with prior RCTs and Cochrane evidence.	Moderate	Strong
Reserve ICSI for clear indications.	Severe male-factor infertility, prior fertilization failure, selected sperm-retrieval cases or laboratory-specific indications.	Low	Strong
Do not use ICSI solely because infertility is unexplained.	WHO guidance and 2025 RCT support no routine benefit.	Moderate	Strong

Sources: [Nature Medicine](#), [WHO guideline PDF](#).

E. Ovulation Induction

A 2026 BMJ Open/PubMed protocol plans a systematic review and network meta-analysis of RCTs comparing letrozole, clomiphene citrate, gonadotropins, combination regimens and expectant management/placebo, with or without IUI, in women aged 18–40 years with unexplained infertility. The primary outcome is live birth per woman randomized [BMJ Open](#), [PubMed](#).

Existing summarized evidence cited in a clinical guideline found that gonadotropins did not improve live-birth rates compared with oral agents when used at low doses or with strict cancellation policies, while higher-dose gonadotropins or lax cancellation policies increased pregnancy but also increased multiple gestations [UnitedHealthcare guideline PDF](#).

Recommendation	Evidence basis	GRADE Quality	Strength
For unexplained infertility, do not assume ovarian stimulation with timed intercourse improves live birth over expectant management.	WHO recommendation; evidence uncertainty.	Low	Conditional
If using ovulation induction with IUI, minimize multiple-gestation risk through	Evidence indicates higher-dose gonadotropins increase	Low	Strong

Recommendation	Evidence basis	GRADE Quality	Strength
low-dose protocols and cancellation criteria.	multiple gestation risk.		
Prefer live birth, multiple pregnancy, OHSS and miscarriage as core outcomes in future ovulation-induction studies.	BMJ Open protocol and infertility trial-reporting priorities.	Low	Strong

F. Male-Factor Infertility

AHRQ lists male-factor infertility options including:

- Treatment of diagnosed endocrinopathy or other conditions affecting sperm production
- Empiric hormonal or other agents
- Varicocele surgery
- IUI
- IVF
- ICSI
- Donor sperm [AHRQ CER PDF](#).

Recommendation	Evidence basis	GRADE Quality	Strength
Evaluate male factors early, including semen analysis and targeted urologic/endocrine assessment where abnormal.	WHO guideline scope and AHRQ treatment framework.	Low	Strong
Treat reversible male-factor causes where identified.	AHRQ lists medical treatment of endocrinopathy and other sperm-production disorders.	Low	Conditional
Use ICSI when severe male-factor infertility is present or when clinically indicated.	ART standard practice; AHRQ recognizes ICSI as an option for male-factor infertility.	Low	Strong
Avoid empiric male-factor interventions without evidence of benefit, especially where cost or burden is high.	Evidence uncertainty across many male-factor interventions.	Low	Conditional

G. ART Management, Ovarian Stimulation and OHSS Prevention

ESHRE's 2025 ovarian stimulation guideline update contains **121 recommendations**, including recommendations on pre-stimulation evaluation, pre-treatment therapies, pituitary suppression, ovarian stimulation, monitoring, triggering final oocyte maturation, luteal support, OHSS prevention, fertility preservation and oocyte donation [PubMed](#).

Recommendation	Evidence basis	GRADE Quality	Strength
Individualize ovarian stimulation protocols based on ovarian reserve, age, prior response, safety and patient preferences.	ESHRE 2025 ovarian stimulation update.	Moderate	Strong
Implement OHSS-prevention strategies in patients at increased risk.	ESHRE update and ASRM current practice documents include OHSS prevention.	Moderate	Strong
Use ART after less invasive treatments fail or when prognosis indicates ART should not be delayed.	WHO stepwise management; ESHRE ART-management guidance.	Low	Conditional

Sources: [PubMed ESHRE update](#), [ASRM Practice Guidance](#).

2.3 Evidence Gaps and Future Research Needs

WHO acknowledges that the 2025 infertility guideline does **not cover all aspects of infertility and fertility care** and that future editions are expected to expand scope [WHO IRIS summary PDF](#).

Priority evidence gaps:

1. Fertility preservation

- WHO identified fertility preservation as an area for future recommendations [WHO news release](#).
- ASRM has a 2026 committee opinion on fertility preservation in patients with medical indications [ASRM fertility preservation committee opinion](#).

2. Third-party reproduction

- WHO identifies third-party reproduction as a future area for guidance [WHO news release](#).
- Policies must protect rights and prevent discrimination [WHO infertility fact sheet](#).

3. Impact of pre-existing medical conditions

- WHO identifies this as a future recommendation area [WHO news release](#).

4. Unexplained infertility

- Comparative effectiveness of expectant management, OS-IUI, IVF and IVF/ICSI remains uncertain, especially by prognosis subgroup [Cochrane](#).

5. Ovulation induction strategy

- The 2026 BMJ Open protocol highlights ongoing uncertainty about letrozole, clomiphene, gonadotropins and combination strategies in unexplained infertility [BMJ Open](#).

6. Male-factor infertility

- More high-quality RCTs are needed for empiric medical therapy, antioxidants, varicocele management subgroups and sperm-function testing.

7. Equity and affordability

- WHO notes that infertility tests and treatments are often paid out-of-pocket, and in some settings a single IVF cycle can cost double average annual household income [WHO news release](#).

8. Implementation research

- Countries need evidence on how to adapt WHO recommendations to different health-system, financing, workforce and cultural contexts [WHO IRIS summary PDF](#).

2.4 Updated Recommendations Based on New Evidence

Original WHO Recommendation / Theme	New Evidence Impact as of 29 April 2026	Updated Recommendation
Integrate fertility care into health systems and financing.	No contradictory evidence; WHO and ASRM continue to emphasize equity and access.	Maintain strong implementation priority. Countries should include infertility care in reproductive-health strategies and reduce catastrophic out-of-pocket spending.
Provide fertility education and prevention interventions.	No new evidence changes the recommendation.	Maintain recommendation. Include fertility awareness, age-related fertility decline, STI prevention and tobacco cessation in primary care and reproductive-health settings.
Give brief tobacco-cessation advice.	WHO-reported GRADE remains moderate certainty, strong recommendation.	Maintain strong recommendation. Tobacco-cessation counselling should be routine in fertility care.
Use expectant management rather than ovarian stimulation with timed intercourse for unexplained infertility.	Cochrane evidence supports uncertainty for active interventions in broad unexplained-infertility populations.	Maintain conditional recommendation. Use prognosis, age, duration of infertility and patient preference to decide duration of expectant management.
Consider IVF after unsuccessful stimulated IUI in unexplained infertility.	Cochrane suggests OS-IUI and IVF/ICSI may both improve outcomes in poor-prognosis couples; superiority of IVF/ICSI over OS-IUI remains uncertain.	Maintain conditional recommendation. IVF is reasonable after failed stimulated IUI, especially with older age, longer infertility duration or poor prognosis.
Prefer IVF alone rather than routine IVF with ICSI when severe male factor is absent.	2025 Nature Medicine RCT strengthens evidence that routine ICSI does not improve live birth	Upgrade confidence in practice direction. Use conventional IVF as default when severe male factor or prior fertilization failure is absent.

Original WHO Recommendation / Theme	New Evidence Impact as of 29 April 2026	Updated Recommendation
	and may not reduce total fertilization failure.	
Manage ovarian stimulation using individualized protocols and OHSS prevention.	ESHRE 2025 update provides detailed ART-management recommendations.	Maintain and operationalize. Use individualized stimulation, monitoring, trigger and luteal-support strategies; prioritize OHSS prevention.
Provide psychosocial support.	No contradictory evidence; WHO continues to emphasize depression, anxiety, stigma and isolation.	Maintain strong recommendation. Psychosocial support should be embedded at diagnosis, during treatment and after unsuccessful treatment.
Encourage research on fertility preservation, third-party reproduction and pre-existing conditions.	ASRM and ESHRE guidance activity confirms these as active areas.	Maintain research priority. Future WHO updates should address these domains with explicit GRADE recommendations.

Practical Clinical Pathway Based on WHO and Updated Evidence

1. Confirm infertility definition and timing

- Evaluate after 12 months of regular unprotected intercourse, earlier when clinically indicated.

2. Evaluate both partners

- Semen analysis.
- Ovulation assessment.
- Tubal and uterine assessment where indicated.
- Review age, duration of infertility, prior pregnancy, comorbidities, medications, STI history and lifestyle risks.

3. Address prevention and reversible factors

- Treat STIs.
- Provide tobacco-cessation advice.
- Counsel on diet, weight, alcohol, physical activity and timing intercourse.

4. Classify infertility

- Ovulatory dysfunction.
- Tubal disease.
- Uterine cavity disorder.
- Male factor.
- Unexplained infertility.
- Mixed factors.

5. Use stepwise, prognosis-based treatment

- Expectant management where prognosis is reasonable.
- Ovulation induction for ovulatory dysfunction.
- IUI or OS-IUI for selected mild male-factor or unexplained infertility.
- IVF when less invasive treatment fails or prognosis indicates ART should not be delayed.
- ICSI only when indicated, not routinely for unexplained infertility or non-severe male-factor cases.

6. Embed psychosocial and equity support

- Discuss cost, access, treatment burden, likelihood of success and alternatives.
- Offer counselling and mental-health support.
- Respect reproductive autonomy and non-discrimination.

Overall Conclusion

As of **29 April 2026**, the WHO 2025 *Guideline for the prevention, diagnosis and treatment of infertility* remains the most current and authoritative global WHO infertility guideline. The strongest post-guideline clinical signal is that **routine ICSI should not replace conventional IVF in patients without severe male-factor infertility**, supported by a 2025 randomized trial in *Nature Medicine* and aligned with WHO guidance [Nature Medicine](#), [WHO guideline PDF](#). For unexplained infertility, evidence continues to support individualized, stepwise management because comparative live-birth evidence remains uncertain [Cochrane](#). WHO's broader message is unchanged: infertility care should be evidence-based, affordable, rights-based, psychologically supportive and integrated into health systems.